

MEDICATION HELPS AND HINTS

1. What is Humulin 70/30

- It is a mix of insulins, R and N.
- 70 and 30 are percentages
- 70% is NPH
- 30% is Regular

Note: How do you remember that 70 is N? 7030 is like a fraction. The top number is called a Numerator which starts with N and so does Insulin NPH.

Examples:

- If you gave 100U of 7030 → there would be 70U of NPH and 30U of Regular
- If you gave 50U of 7030 → there would be 35U of NPH and 15U of Regular

2. Can you mix insulins in the same syringe? Yes.

How?

You want to be an RN right? Then do it in that order. Regular then NPH. Clear then cloudy.

Pressurizing the vile?

Inject the air into the N first then inject the air into the R. Then draw up the R then draw up the N.

3. Injections

- They will give you the name of the injection but ask what needle you need to use:

EXAMPLE: If you're getting an IM injection, what needle will you use?

- a) 21 gauge 5/8 inch
- b) 22 gauge 1/8 inch
- c) 21 gauge 1 inch
- d) 25 gauge 5/8 inch

CLUE:

The clue is in the abbreviation:

- IM. Look at the first letter. I looks like 1. Therefore, choose the answer that has both 1's in them.
- If you're getting a SC injection. Look at the abbreviation, SC. S looks like 5. Choose an answer that has 5 in both parts. Therefore, d.

4. Heparin versus Coumadin

	Heparin	Coumadin
Route	IV or SC	PO only
Therapeutic Action Onset	Works immediately	Takes a few days to a week to work
	Cannot be given for longer than 3 weeks (except for Lovenox) because in 21 days you start to make heparin antibodies	Can be given for the rest of your life
Antidote	Protamine Sulfate "heParin → Protamine"	Vitamin K "Koumadin"
Lab Test	PTT Heparin = 7 fingers 3 fingers left = PTT	PT which is what the INR is derived from Coumadin = 8 fingers 2 fingers left = PT
Pregnancy	Can be given to pregnant women	Cannot be given to pregnant women

What's the only major tranquilizer that can be given to pregnant women? Haldol.

5. K Wasting Diuretics

- If it ends in X or + DIURIL → X's OUT K → it is POTASSIUM WASTING
- If it does not end in x and is not a diuril → it is POTASSIUM SPARING

"-SEMIDES are typically X's"

6. Baclofen and Flexaril

Boards tests about these muscle relaxants:

2 Side Effects:

1. Drowsiness/Fatigue
2. Muscle weakness

3 Teachings:

1. Don't drink
2. Don't drive
3. Don't operate heavy machinery

"Baclofen: If I'm on my back loafin, I'm on my baclofen" IT'S A MUSCLE RELAXANT.

FIRST STAGE OF LABOUR AND DELIVERY → LABOUR

Know the whole chart.

	LATENT	ACTIVE	TRANSITIONAL
DILATION	0 – 4 cm	5 – 7 cm	8 – 10 cm
FREQUENCY OF CONTRACTIONS	5 – 30 minutes	3 – 5 minutes	2 – 3 minutes
DURATION OF CONTRACTIONS	15 – 30 seconds	30 – 60 seconds	60 – 90 seconds
INTENSITY OF CONTRACTIONS	Mild	Moderate	Strong

The way they test their knowledge:

Question: 5cm dilated, 5 minutes apart, last for 45 seconds. What PHASE is she in? ACTIVE.
You need to know the numbers to put them in the right one.

Three column, sequential table. There's 3 columns, the other leaves off, the next one picks up.
Easiest way to master is to memorize the MIDDLE column (active).

Note: Contractions should not be longer than 90 seconds or closer than 2 minutes.

How will they ask this?

- How do you know a woman is in trouble. For instance, they will give you four women and ask, who is in trouble?
- How do you know a woman is in uterine tetany?
- How will you know a woman is in uterine hyperstimulation?
- What parameters would make you stop ptosin?

ASSESSMENT OF CONTRACTIONS

Teach mom how to do this!

Frequency: Beginning of one contraction to the beginning of the next contraction.

Duration: Beginning to end of one contraction.

Intensity: The strength of contraction which is purely subjective.

*** Teach her to palpate with ONE HAND over the fundus with the PADS of the fingers.**

THREE COMPLICATIONS YOU NEED TO KNOW

1. PAINFUL BACK LABOUR (Usually presentation is OP think "O PAIN") → LOW PRIORITY

- **Tx: POSITION THEN PUSH**

- 1. POSITION KNEE CHEST → goes on hands and knees with rear end up and head down which brings baby off the sacrum and coccyx
- 2. PUSH → take your fist and push on sacrum; counter-pressure and relieves pain

2. PROLAPSED CORD → **HIGH PRIORITY**

- This is bad because baby can kill itself
- Cord is coming out first
- Baby comes out and presses on cord → dies
- **Tx: PUSH THEN POSITION**
 1. PUSH the head back up
 2. POSITION knee chest
- Stay that way until C-SECTION performed
- You are riding on the cart with her.
- You will push then position because delaying pushing in order to position would increase the risk to the baby.

3. INTERVENTIONS FOR ALL OTHER COMPLICATIONS OF L&D (There's probably 16 of them)

* TREAT WITH "**LION PIT**"

L – Turn them on their left side

I – Increase IV

O – Oxygenate them

N – Notify MD

PIT In a crisis, if ptosin is running, STOP IT. Could this be the first thing you would do?
Yes, then LION.

PAIN MEDICATION FOR WOMEN IN LABOUR

Do not administer pain medication to a woman in labour if the baby is likely to be born when the medication PEAKS.

Question: You have a primigravida (first timer) at 5cm, who wants her IV push pain med. Will you give it to her, or not?

Is it likely that a primigravida at 5cm is gonna deliver in the next 15-30 minutes after you give it (we learned IV meds peak at 15-30 mins)?

NO! So yes, you can administer.

Question: You have a multigravida at 8cm, who wants her IM pain med? Will you give it to her or not?

Is it likely that a multigravida at 8cm is gonna deliver in the next 30-60 minutes after you give it (we learned that IM meds peak in 30-60 mins)?

YES! So no, do not administer.

FETAL MONITORING PATTERNS

7 that you need to know:

Pattern	Classification	Interventions
Low Fetal HR < 110 bpm	Bad	LION Left side IV O2 Notify If PIT running → STOP
High Fetal HR > 160 bpm	Good	Document and take mom's temperature Mom probably has fever
Low baseline variability FHR stays the same and does not change In other words, stable VS	Bad	LION Left side IV O2 Notify If PIT running → STOP
High baseline variability FHR always changing In other words, VS are not stable	Good	Document it
Late decelerations HR slows down near end of contraction or after	Bad	LION Left side IV O2 Notify If PIT running → STOP
Early decelerations HR slows down before contraction or at the beginning of one	Good	Document it
Variable decelerations "VERIABLE VERY BAD" THIS IS PROLAPSED CORD	VERY BAD	PUSH THEN POSITION Push baby's head Position knee chest

Note: Once baby is born and VS are the same = GOOD
Before baby is born and VS are the same = BAD

Therefore:

1 Very bad: Variable decelerations

3 Bad: Start with L which you do Lion. L and L.

3 Good: Document it. If it doesn't start with L, it's good.

“VEAL CHOP”

PATTERN	CAUSE
Variable decelerations	Cord compression
Early deceleration	Head compression
Acceleration	Okay
Late deceleration	Placental insufficiency

ACE OF SPADES

They're always winners. In OB, there's an answer that always wins.

CHECK FETAL HEART RATE! NO MATTER WHAT.

SECOND STAGE OF LABOUR → DELIVERY OF BABY

ORDER

1. Deliver the head
2. Suction the mouth then suction the nose (alphabetical mouth then nose)
3. Check for nuchal cord
 - a. Nuchal: Around the neck
4. Deliver the body
5. Baby must have an ID band on before it leaves the delivery area

THIRD STAGE OF LABOUR → DELIVERY OF PLACENTA

1. Make sure it's all there!
2. Check that the cord has 3 vessels.
 - a. How many of each? Two arteries. One vein. Think of woman's name AVA.

FOURTH STAGE OF LABOUR → RECOVERY (First 2 hours after the delivery of placenta.)

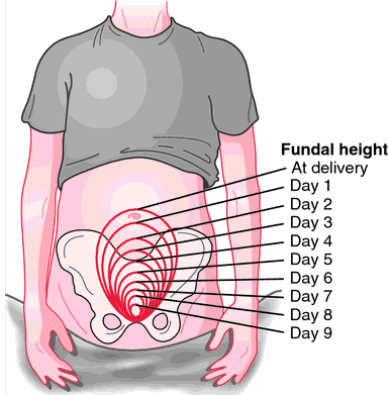
Four things you do, four times an hour (q15mins), in the fourth stage! 4x4x4

1. Vital signs: Ax for shock
 - a. Pressures go down
 - b. Rates go up
 - c. Pale, cold, clammy
2. Fundus: Check the fundus
 - a. Boggy → massage
 - b. Displaced → catheterize
3. Pads: How much she's bleeding
 - a. Excessively → saturate pad in 15 minutes or less; that's why you check every 15 minutes
 - i. **Question:** It's 98% saturated, is she in trouble or not?
 - ii. She's NOT in trouble because it has to be 100% saturated.
 - b. Put on a new pad q15mins
4. Roll her over
 - a. Check for bleeding underneath her

POSTPARTUM ASSESSMENT

Ax every 4 – 8 hours; depending on if she's stable or not

Ax **BUBBLE HEAD**:

ASSESSMENT		INTERVENTION
B	reast	
U	uterine Fundus	Boggy → Massage If not midline → Catheterize Ax: Height of fundus in relation to belly button - Fundal height = day post partum - You want it MIDLINE!!! Example: In 4th post partum day, point and click... 
B	ladder	
B	owel	
L	lochia	Occurs in this order: Rubra → If you rub and rub and rub something → RED. Serosa → “ROSA” → if your cheeks are rosey, they are → PINK. Alba → If you are an ALBino → WHITE. <u>Amount:</u> 4 - 6 inches on a pad is normal Excessive bleeding is saturated pad in 15 minutes or less
E	episiotomy	
H	hemoglobin and Hematocrit	
E	extremity Check	Ax for THROMBOPHLEBITIS - Best answer: Bilateral calf circumference measurement - NOT Homan's sign*
A	effect	Emotional
D	discomfort	

Therefore, 3 big things are **FUNDUS**, **LOCHIA**, and **EXTREMITY CHECK**.

VARIATIONS IN THE NEWBORN

All of these things are NORMAL:

1. **Milia:** Distended sebaceous glands which appear as tiny white spots on baby's face.
2. **Epstein's pearls:** Small, white epithelial cysts on baby's gums.
3. **Mongolian spots:** Bluish/black macules appearing over the buttocks and/or thighs of darker skinned neonates.
4. **Erythema toxicum neonatorum:** Red papular rash on baby's torso which is benign and disappears after a few days.
5. **Hemangiomas:** Benign tumor of the capillaries.
6. **Cephalohematoma:** Swelling caused by bleeding between the osteum and periosteum of the skull. This swelling does not cross suture lines.
7. **Caput Succedaneum:** Edematous swelling on the scalp caused by pressure during birth. This swelling may cross suture line. It usually disappears in a few days.
 - a. Initials are CS → CROSSES SUTURES → CAPUT SYMMETRICAL
8. **Hyperbilirubinemia:** Normal, physiologic jaundice appears after 24 hours of age and disappears at about one week.
9. **Vernix caseosa:** Whitish, cheese-like substance which covers the skin on an unborn baby.
10. **Acrocyanosis:** Normal cyanosis of the baby's hands and feet which appears intermittently over the first 7-10 days.
11. **Nevus/Nevi:** The generic term for a birthmark.

OB MEDS

Know what they are and what they do

TOCOLYTICS: STOP LABOUR (STOP)

1. Terbutaline
 - a. Causes maternal tachycardia
2. Magnesium Sulfate
 - a. Give a lot → HYPERMAGNESEMIA → Everything does the opposite; uterine contractions GO DOWN but so will everything else
 - b. Bradycardia
 - c. Hypotension
 - d. 1+ reflexes (go down)
 - e. Bradypnea
 - f. Dec LOC

* Top 2 that boards check knowledge → **RESPIRATORY AND REFLEXES!**

Therefore, **ASSESS RESPIRATIONS AND REFLEXES**

Parameters of Titration:

Respirations: If respirations are at least 12 it's okay. If it's below 12, you need to slow it down.

Reflexes: You want 2+. If it's 1+, slow it down. If it's 3+, speed it up.

OXYTOXICS: STIMULATE AND STRENGTHEN (START)

1. Ptosin
 - a. Can cause uterine hyperstimulation
 - i. Longer than 90 seconds
 - ii. Closer than every 2 minutes
 1. STOP THE PIT
2. Methergine
 - a. Causes HTN → because it causes VC of BV

Note: Methergine and Magnesium Sulfate both start with M and are used in OB. One of them lowers BP and one of them elevates BP.

FETAL LUNG MATURING MEDICATIONS: TO MAKE FETUS' LUNGS MATURE FASTER

1. Betamethasone (steroid)
 - a. Given to mother
 - b. Given IM
 - c. Given BEFORE baby is born

Note: You can repeat it if the baby is in utero

2. Servanta (surfactant)
 - a. Given to neonate
 - b. Given TRANSTRACHEAL (blown in through trachea)
 - c. Given AFTER baby is born